WholeMind
RESILIENCE

The Case for WholeMind Resilience

Evidence of Community Need — Public Survey Findings

Anonymised survey of 88 members of the Norfolk public, conducted ahead of launch. Prepared August 2026.

99% experienced high stress, burnout or wellbeing difficulty in the last 12 months	**91%** already know mindfulness supports mental wellbeing	**88-89%** likely or very likely to use mindfulness coaching, group sessions or condition-specific courses
72% already pay, or would consider paying, for wellbeing support that felt valuable	**35%** identify as having a disability	**51%** carry caring responsibilities for others
62% identify as having a long-term health condition (LTC)	**96%** of those with a long-term condition would use condition-specific courses	**100%** of people living with an LTC experienced stress or burnout in the last year

1. Why we asked

Before launch, WholeMind Resilience surveyed members of the public, predominantly across Norfolk, to test whether the need we believe exists is genuinely felt by the people we intend to serve, and to understand what kind of support people actually want, not just what we assume people want.

We wanted to start right, by co-designing the organisation, and the services, with people in the community. By listening openly to what would be genuinely helpful to people, and using the responses to inform our decisions about where we start, and what we deliver.

This document sets out what 88 respondents told us, in their own numbers and, in places, their own words. All responses are anonymised. No names, email addresses, or full postcodes appear anywhere in this document or in the data behind it, location detail is reported only at town or postcode-area level, consistent with WholeMind's data handling policy.

2. The scale of the problem

Respondents were asked whether they, or someone close to them, had experienced any of a list of three difficulties in the past 12 months. Almost all respondents (or someone close to them) had experienced at least one of the three difficulties in the last 12 months. The results leave little doubt that stress, burnout and difficulties with mental wellbeing is a live, current need, not a hypothetical one.

Only a single respondent (1%) reported experiencing none of these difficulties. This is a self-selected sample rather than a random population survey (see Section 11 for some of the limitations). However, these results are in line with other surveys such as that from the Mental Health Foundation [1] that found in the last year of the survey 74% of people felt so stressed that they were having difficulty coping and in the NHS workplace stress impacts 42% of staff [2]. The near-universal prevalence of stress, burnout or wellbeing difficulty among people who chose to engage with this survey is itself a strong signal: the people WholeMind is designed for are already living the problem WholeMind exists to address.

3. Awareness of mindfulness as a solution

Survey respondents are aware of mindfulness as an intervention to mitigate some of the difficulties. 91% of respondents already understand that mindfulness can support mental wellbeing.

Awareness level	% of respondents
Already taught mindfulness before	50%
Aware it's beneficial, not yet taught	41%
Not aware mindfulness supports wellbeing	9%

This matters for how WholeMind positions itself: the barrier for most people isn't awareness or scepticism, it's access. Four in ten respondents are aware of the benefits of the tool but have not yet been taught; what's missing is a trusted, accessible route to it.

4. Demand for specific services

Respondents rated how likely they, or someone they know, would be to use each service WholeMind is considering offering locally. We focused on services that could be included in the early launch phase, rather than the wider range that may be added later through partnerships:

Service	Likely or very likely to use
Mindfulness or meditation group sessions	89%
Courses for specific conditions (e.g. menopause, long-term illness, cancer)	89%
Mindfulness coaching	88%
Mindfulness tuition & courses	85%
Mindful art groups	69%
Creative journalling groups	64%

Social paint events	60%

The survey suggests that at least six in ten of respondents would be likely or very likely to use one or more of the services (60% for social paint events), and that for the three highest-scoring options (group sessions, condition-specific courses, and 1:1 coaching) 88-89% of respondents would use these services. This gives WholeMind's launch offer (mindfulness tuition, meditation sessions, and coaching) direct evidential backing, and also suggests condition-specific courses could be an early opportunity.

5. Long-term health conditions: a case for health commissioning

Condition-specific courses (e.g. for menopause, long-term illness, or cancer) was one of the seven services tested in Section 4, and it tied for the highest demand of all of them (89%). In England about 46% of the population aged 16 and over have at least one long term condition [3]. Prevalence is slightly higher in this survey where 62% of respondents identify as having a long-term health condition. This shows that this isn't a marginal interest group within the data and it speaks directly to WholeMind's ambition to secure NHS and public health commissioning in future. WholeMind will give people the tools to build mental health resilience to help manage their long term conditions.

Among respondents with a long-term health condition (62% of all respondents)	Result
Likely or very likely to use condition-specific courses	96%
Experienced high stress, burnout or wellbeing difficulty in the last 12	100%
Say affordability would stop them accessing wellbeing support	18%

In this survey all respondents with a long term condition have experienced high stress, burnout or wellbeing difficulty in the last 12 months, and almost all would be likely or very likely to use condition-specific courses. Free-text comments provide additional information to understand how courses could be adapted to meet needs: respondents specifically raised caring for a partner with Alzheimer's, living with learning disabilities, anxiety, depression and attachment disorder, and asked for delivery adapted to autistic adults and people who are hard of hearing. This is a population already managing significant, ongoing health burden, already experiencing stress and burnout at a near-universal rate within this sample, and already telling WholeMind directly what would need to change for the offer to work for them.

For commissioning purposes, this is the strand of evidence most directly relevant to future NHS, ICB or public health conversations: a locally-evidenced population managing long-term conditions, at very high risk of stress and burnout, highly receptive to non-clinical preventative support, and including a meaningful group (18%) for whom cost is a genuine barrier. This is the information a commissioner needs to see to help understand a business case for funding a preventative service alongside clinical care. This should be treated as a standing reference point whenever WholeMind explores NHS or public-sector commissioning routes, alongside the separate work for DSPT accreditation data-governance assurance.

6. How people want to access support

Norfolk is a rural county, with areas of significant deprivation, and where about 20% of the population live in a coastal community [4]. We asked about delivery preferences to understand how these barriers would impact accessibility of programmes, some of which could be delivered online. Around six in ten respondents want flexibility, in-person or online, depending on location, rather than a single fixed format. A further third want in-person delivery specifically. About 2% want online-only delivery. This highlights that face-to-face delivery remains an important anchor for the public offer, alongside an online option.

People are willing to travel to access support. Of those who are willing to travel, 48% would be willing to travel three miles within the local area, 40% would travel up to 10 miles and 12% of respondents would be willing to travel further than 10 miles. This highlights the case for a genuinely local, multi-venue delivery model across Norfolk, rather than a single central hub.

Geography

Over half of respondents (58%) gave a Norwich-area (NR) postcode, with the remainder spread across other towns including Fakenham, North Walsham, Watton, Beccles, and the surrounding villages. This highlights the need for support across different geographic areas, from city environments to market towns, to villages. The need is not just concentrated in the city of Norwich.

As the data was collected through surveys posted in 102 local community groups on social media, we note that the impact of social media algorithm behaviour in each group is unknown.

7. Affordability and the case for funded places

72% of respondents already pay, or would pay, for wellbeing support that felt genuinely valuable. This is evidence that a paid service tier is commercially viable, not just a hoped-for assumption. However, 17% said affordability is a genuine barrier, and 26% of all respondents sit in the lowest household income bracket (under £20,000). Interestingly this proportion of low income households in this survey is much higher than national estimates. Information from the IFS indicates that in 2022 about 10% of households in the UK had an income of £20,000 or less [5]. This shows that WholeMind's offer of support is likely to be used by those experiencing income deprivation and may help to address inequality in outcomes for these populations.

Payment attitude	% of respondents
Already pay for wellbeing support	38%
Would consider paying if the service felt valuable	34%
Cannot afford to pay for wellbeing support	17%

Read together, this is direct evidence for WholeMind's dual-tier model: a paid service that most people are willing to fund, alongside dedicated funded places for those who experience inequality. This approach to proportionate universalism is reflected in WholeMind's social pledge.

8. Who responded

There are a few things to note about the characteristics of the population that completed the survey.

Characteristic	Finding
Age	45% aged 26–49, 42% aged 50–65, 9% over 65, 2% aged 18–25
Gender	87.5% women, 9% men, 1% trans man, remainder other/unstated
Disability	35% identify as having a disability
Long-term health condition	62% identify as having one
Caring responsibilities	51% carry caring responsibilities for others
Household income	26% under £20,000

The proportion with disability (35%) and the proportion with one or more long-term health condition (62%) are notably high. For comparison, about just over 20% of Norfolk residents are disabled under the equality act [6]. The previous sections highlight that this is a population that already needs, and would use, accessible and adapted delivery, not a generic one-size offer.

The survey remains heavily skewed toward women (87.5%), and the sample continues to substantially under-represent men. Very few 18–25 year-olds responded.

We anticipate the lack of young voice to be due to our method. Facebook is notably less used by young people. The lack of voice from males of all ages is more concerning when we consider the statistics for male mental health. WholeMind has reached out to a local organisation supporting men to seek a better understanding of why the survey did not appeal to men.

9. What people told us directly

More than a quarter of respondents (30%) added their own comments. Common, recurring themes included:

- Accessibility and adaptation: several respondents specifically asked for delivery adapted to neurodivergent people, autistic adults, and people who are hard of hearing. Highlighting that traditional mindfulness formats don't work for everyone.
- Life-circumstance-specific groups: requests for support around anticipatory grief and bereavement, caring for a partner with dementia, and support for adult children of ageing parents.
- Age gaps: a specific call for something aimed at 16–25 year-olds, and a general wish for the offer to reach all ages and both genders.
- Practical delivery preferences: interest in outdoor and nature-based sessions (walking groups specifically mentioned), and a gap in provision during school hours for parents/carers.
- Activity-based approaches for men: one respondent, who had recently retired early due to burnout and stress, suggested that groups combining physical activity with mindfulness — walk and talk sessions, being in nature, or task-focused activities such as kayaking — may appeal more to men than traditional seated mindfulness formats. This is a useful, concrete lead for WholeMind's continuing work to understand why the survey has not yet reached men in similar numbers to women.
- How WholeMind differs from existing local or online mindfulness offers: this is more of a question. Why is WholeMind different from other offers and how have people with lived

experience of struggling with mindfulness been involved in shaping it? Something we have to answer.

- Practitioners and service users are willing to get involved: existing local practitioners (coaches, hypnotherapists, a former Mind peer support worker, a physiotherapist setting up locally) expressed interest in getting involved or partnering with WholeMind. This is a signal of community appetite beyond service users alone, relevant to WholeMind's volunteering and partnership plans.

10. Willingness to get involved: volunteering and organisational interest

Beyond demand for WholeMind's own services, the survey also asked if people would be willing to support delivery directly, not just attend it.

87%

would consider volunteering at group events or sessions (40% yes, 47% maybe)

This is a strong signal that community capacity exists alongside community need. This is relevant to WholeMind's volunteering plans, and to any funder assessing whether the organisation can extend its delivery capacity beyond the founder alone.

This appetite isn't just hypothetical. As a direct result of the survey, five organisations approached WholeMind independently to discuss how they might support or partner with the CIC. These include:

- A non-clinical wellbeing organisation
- A clinical therapy CIC
- An independent coach
- A physiotherapist
- A CIC working with communities across a variety of needs

Taken together, the volunteering figures and this direct organisational response suggest WholeMind's launch is being met with genuine appetite to collaborate, not just to attend. This highlights that there is need, demand and delivery capacity in the form of local partnerships to meet the demand and address the need.

11. Limitations of this survey

In the interest of transparency, and to ensure we use an honest evidence base, not an oversold one:

- This is a self-selected convenience sample of 88 people, not a random or statistically representative survey of the Norfolk population. However, the responses indicate genuine, strong signals of need and demand.
- Respondents skew heavily female (87.5%) and the findings may under-represent men's needs, interest, and may reflect barriers to engagement. This remains a substantial skew despite a small improvement since the original 82-response sample (90.2% female), and should not be read as a resolved limitation.
- Respondents skew toward ages 26–65; 18–25 year-olds remain almost entirely absent, and over-65s are a relatively small share.

- Geography is concentrated in and around Norwich and mid-Norfolk market towns. There are some respondents from other areas including more rural areas but the number of respondents is low from other areas.
- Some comparisons with Norfolk and national figures have been included but a more detailed comparison would highlight the likely market size and population that would benefit from WholeMind's offer. This will be done as part of the market-sizing work.

12. What this shows

Taken together, this survey gives WholeMind Resilience an initial, primary evidence base, not just sector-level statistics, for three core claims:

The need exists: the need is real and current among the people surveyed (99% affected)

Demand for mindfulness services exists: the proposed service response would be used (85–89% demand across core services)

A mixed paid / funded model is likely to work: a mixed paid/funded model is the right structure to reach everyone who needs it (72% willing to pay, 17% priced out)

It also surfaces specific, actionable gaps; accessibility for neurodivergent and disabled people, provision for 16–25 year-olds, and a clearer answer to 'how are you different', that WholeMind can now address deliberately rather than discover later.

Importantly, it shows something beyond need, that there is a real, current appetite from both individuals and organisations to get involved in delivering the response, not just receiving it.

**The WholeMind Resilience public survey remains open whilst we address demographic gaps and this report may be updated at a later stage.* https://forms.cloud.microsoft/r/fwAXRDghsh

Sources

- [1] Mental Health Foundation, stress statistics: https://www.mentalhealth.org.uk/explore-mental-health/statistics/stress-statistics
- [2] NHS Staff Survey 2025, National Results Briefing: https://www.nhsstaffsurveys.com/static/98fbf017ea18aad1523254e02072cd02/National-Results-Briefing-2025.pdf
- [3] NHS Digital, Health Survey for England 2024, Adults' Health: https://digital.nhs.uk/data-and-information/publications/statistical/health-survey-for-england/2024/adults-health#key-findings
- [4] Norfolk Insight, JSNA document library: https://www.norfolkinsight.org.uk/jsna/document-library/
- [5] Institute for Fiscal Studies, Where do you fit in?: https://ifs.org.uk/tools_and_resources/where_do_you_fit_in
- [6] NOMIS, Census 2021 report: https://www.nomisweb.co.uk/sources/census_2021/report?compare=E10000020